

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding reasoning paths :

- 1 . In attention and hyperactivity -> ADHD diagnosis -> Behavioral therapy
- 2 . In ability to complete tasks and impulsivity -> ADHD diagnosis -> Behavioral therapy
- 3 . Aggressive behavior and academic challenges -> ADHD diagnosis -> Behavioral therapy

Reasoning Process :

- 1 . **Symptoms Analysis**: The child exhibits inattention , inability to complete tasks , impulsivity , hyperactivity , and aggressive behavior . These symptoms align closely with Attention - Deficit/Hyperactivity Disorder (ADHD) criteria .
- 2 . **Behavioral Issues**: The child's inability to listen , frequent interrupting , and disruptive behavior in school are indicative of difficulties with impulse control and attention , which are hallmark features of ADHD .
- 3 . **Developmental History**: The child has met all developmental milestones , and there is no history of significant neurological or genetic syndromes , making conditions like Rett syndrome or other developmental disorders less likely .
- 4 . **Differential Diagnosis**: While conditions like Angelman syndrome or intellectual disability could present with some similar symptoms , the presence of hyperactivity and impulsivity , along with the absence of other indicative features (e.g ., developmental regression , atypical facial features) , makes these less likely .
- 5 . **Next Step in Treatment**: Given the suspected diagnosis of ADHD , the most appropriate next step is to implement behavioral therapy . This approach focuses on teaching the child strategies to manage impulses , stay focused , and improve listening skills .

Conclusion :

Behavioral therapy is the most appropriate next step in treatment for this child's symptoms , which are consistent with a diagnosis of Attention - Deficit/Hyperactivity Disorder (ADHD) .

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<answer>

Behavior therapy

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